One Drug, Many Prices:
A Proposal to Reform US Healthcare Pricing

Introduction

The US healthcare pricing system is broken: the same product is sold to different payers for wildly different prices, pharmacy benefit managers collect undisclosed sums and hide the amounts as a tightly guarded secret, and the largest public insurer is effectively barred from using its leverage to reduce prices. Americans usually do not know the price at the time of receiving care. This opacity is a structural driver of the price dispersion. No other developed nation operates a pricing system this opaque.

I propose three reforms: consolidated government negotiation, Bloc-wide pricing as a condition of market access, and a central price lookup system. Each has been tried or proposed in isolation with promising results. I show how the three could be combined into a comprehensive plan (see FAQ 1).

Figure 1 shows the payment gap across ten common drugs, hospital services, and outpatient procedures. For example, a colonoscopy costs $880 at the VA, $1,760 under Medicare, $5,816 under a commercial insurer, and $19,206 for uninsured patients. This is not an anomaly. In every case, the price swings several-fold across payers, and the uninsured, a vulnerable group, pay the most.

Two forces determine an insurer's ability to reduce prices: its size, and the availability of alternatives. A large insurer has more leverage because losing its volume hurts providers more, but if providers can walk to another insurer offering higher rates, that leverage evaporates. Leverage is maximized when the insurer is large and irreplaceable. The three reforms below are designed around this logic. Consolidated negotiation makes the government a single, large insurer. Bloc-wide pricing makes it difficult for providers to replace the Bloc. A central price lookup system ensures the negotiated prices are honored, not undercut by secret side deals.

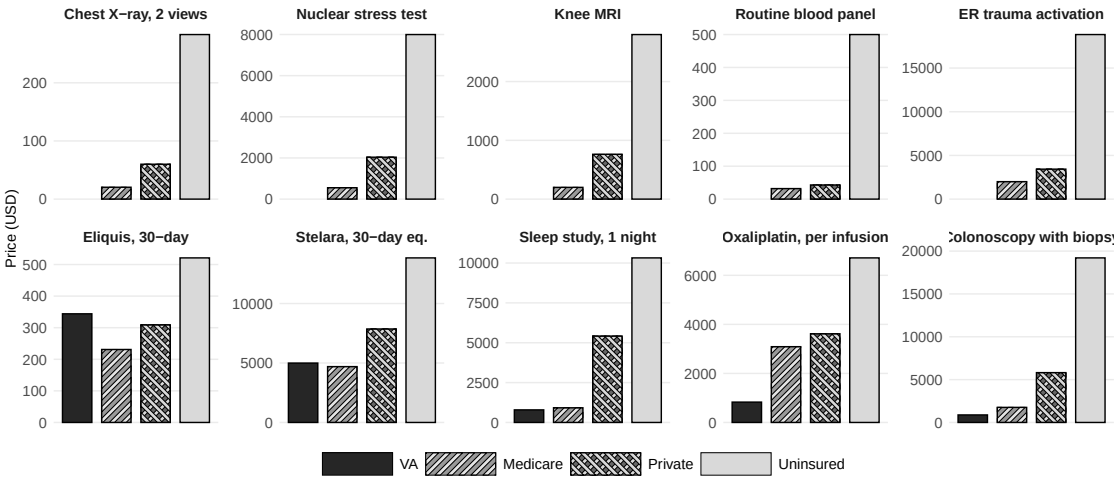

Figure 1: Price gaps across payers for the same service or product.

Notes: The "private insurer" bar is the total negotiated rate, including patient cost-sharing; the "uninsured" bar is the chargemaster or list price. Each panel uses its own vertical scale. Missing bars indicate that a reliable figure was not available in the cited sources. Full sources and derivations are on the companion site.

Reform 1: Consolidated government negotiation

Reform 1 pools all federal healthcare purchasing into a single negotiating Bloc that bargains on behalf of every federal program.

Today, the federal government fragments its purchasing power across Medicare, Medicaid, and at least nine other programs, each with its own formulary, fee schedule, and leverage. For prescription drugs alone, these programs account for 51% of US spending, yet they compete against one another instead of bargaining as one.[1] Hospital and physician payments are just as fragmented. Consolidating these programs into a single negotiating Bloc would use leverage the government has but wastes. Other developed nations leverage their full size in what resembles a bloc, and pay far less.[2]

For most of its history, Medicare was statutorily prohibited from negotiating drug prices, a restriction unparalleled in the developed world. The Inflation Reduction Act of 2022 began to reverse it, authorizing Medicare to negotiate prices for ten high-expenditure drugs. Confining negotiation to ten drugs underutilizes government leverage and wastes taxpayer money. The VA, unencumbered by that prohibition, pays half of Medicare Part D despite being much smaller.[4–6]

Figure 2 compares prices in the US and six other developed countries for nine of those ten drugs. The US list price towers over what every peer nation pays. Even after rebates and discounts, private insurers in the US pay multiples of what patients in Canada, France, or the UK pay for the same drug. Medicare's newly negotiated prices are 22% lower[3] and narrow

the gap but remain well above other nations.

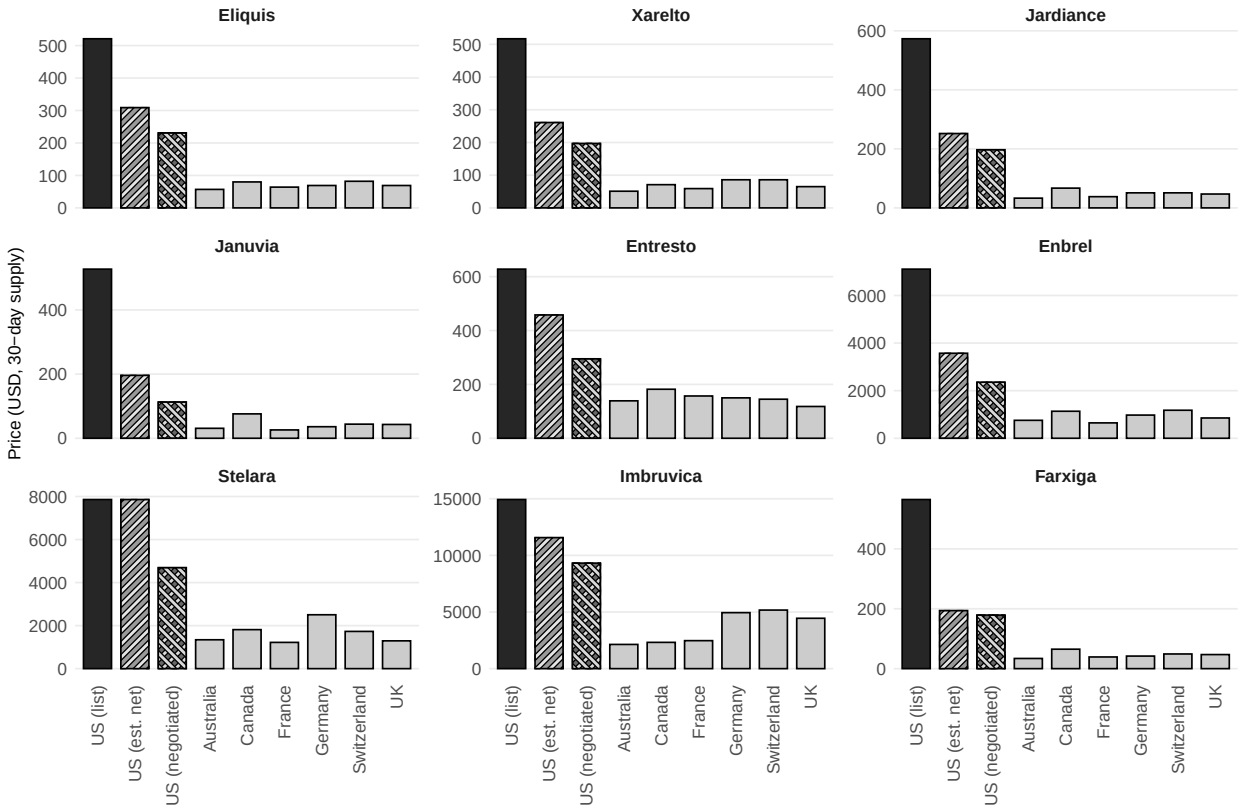

Figure 2: Drug prices by country.

Notes: The solid dark bar is the US list price (paid by the uninsured). The striped bar is the estimated US net price across all US payers combined, after rebates and discounts.[7] The crosshatched bar is the Medicare negotiated price under the Inflation Reduction Act (effective 2026). Insulin aspart is excluded due to reporting incompatibility across countries. Each panel uses its own vertical scale. Full figures and sources are on the companion site.

Reform 1 outlaws rebates, chargebacks, spread pricing, and side payments of any kind. Negotiation is conducted by salaried Bloc negotiators with expertise in pharmacology, clinical medicine, and health economics, who also make formulary and tiering decisions. Bloc negotiators are prohibited from accepting any payment beyond their government compensation package, and for twelve months after leaving the Bloc are barred from employment in any sector of the regulated healthcare industry. Compensation follows the federal Senior Executive Service model: a fixed salary at the SES level and a performance bonus capped at 10% of base, awarded on periodic peer-reviewed evaluations rather than per-contract outcomes (see FAQ 2).

Smaller-scale precedents exist. The Federal Supply Schedule publishes VA-negotiated contract prices that the Department of Defense (including Tricare), the Indian Health Service, the Federal Bureau of Prisons, and the Public Health Service all purchase off,[8] demonstrating that one body can negotiate for many federal agencies. At the state level, pooled purchasing

coalitions such as the Sovereign States Drug Consortium extract supplemental Medicaid rebates of 5 to 15% beyond federal levels by combining state purchasing power.[9,10]

Reform 2: Bloc-wide pricing as a condition of market access

Any provider that contracts with the Bloc retains the freedom to choose which private insurers to contract with, but the contracted price must be the same as the Bloc-negotiated rate. Uninsured patients cannot be refused or charged a different price. The negotiated price is locked in: no insurer can negotiate further discounts on the side. Any provider that violates these terms loses its contract with the Bloc, and with it, access to the majority of the market.

Reform 2 does not make insurance plans identical; it standardizes the price, not the coverage. Once the price of a service is fixed, insurers would compete on what fraction of that price they cover and what fraction the patient pays out of pocket. Generous plans leave the patient with a small copay, cheaper plans with more, but both pay the provider the same.

The case against price discrimination across payers has been argued in the US health-policy literature for decades, most prominently by Reinhardt,[11] and Maryland has run an all-payer hospital system since 1977. Conditioning federal market access on pricing is also well-established: the 340B Program requires Medicaid manufacturers to sell to safety-net providers at 25 to 50% below list, and the Medicaid Best Price rule requires manufacturers to offer Medicaid the lowest price offered to any non-federal purchaser.

Reform 3: A central price lookup system

Reforms 1 and 2 produce one negotiated price per provider per service. Reform 3 makes that price visible. The Bloc operates a public price lookup system, the canonical record of every contracted price in the country. Each entry pairs a service code with a provider and a price. It updates whenever a contract is signed or revised.

Every Bloc contract pre-sets the price of every covered service, whether per service or per bundled episode. Bloc officials directly transfer prices from contract to database, with no provider input. This sidesteps the misreporting, gaming, and non-compliance that become systemic whenever transparency depends on thousands of providers posting their own data.

The lookup system is open, free, and built for comparison. Patients can compare providers within a chosen radius before scheduling care, and physicians, better informed and more influential in directing demand, can route patients to lower-priced providers. A public API lets third parties integrate prices into comparison tools, electronic health records, and patient

portals.

Price transparency is a known driver of price reductions (see FAQ 1), but only if the posted prices are the actual prices and if transparency is enforced. Existing federal transparency rules have failed on both counts: they require posting in formats too inconsistent to compare, and punish non-compliance with symbolic penalties. Under Reform 3, the Bloc posts actual prices, and overcharged patients can file a complaint. Deterrence requires large fines, for example a multiplier on the order of 100 times the overcharge (see FAQ 3), and contract termination for repeat offenders.

Final word

The reforms would benefit patients most. They would pay less, comparison-shop, and know prices before receiving care. As underlying prices fall, insured patients would face lower premiums, and uninsured patients would no longer be overcharged. The US pays multiples of what every other developed country pays for the same drugs, services, and procedures, for no reason beyond institutional inertia. The tools to change that are not exotic. They are familiar, tested at smaller scales, and in partial federal use. What is missing is the decision to put them together.

The online FAQ answers a set of common questions about the proposal. Comments, criticisms, and suggestions are welcome through this link.

References

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